

1. Administrative & Study Identification

Full Study Title: A Study of PF-08046054/SGN-PDL1V in Advanced Solid Tumors **Official Title:** A Phase 1 Study of PF-08046054/SGN-PDL1V in Advanced Solid Tumors **Short Title/Acronym:** SGNPDL1V-001 / C5851001 **Protocol Number:** C5851001 **NCT Number:** NCT05208762 **Sponsor/Company:** Pfizer **Investigational Product:** PF-08046054 / SGN-PDL1V (Vedotin antibody-drug conjugate targeting PD-L1) **Phase of Development:** Phase 1 **Medical Monitor:** Pfizer Clinical Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety, tolerability, and pharmacokinetics (PK) of PF-08046054/SGN-PDL1V (alone and in combination with pembrolizumab) in adults with select advanced solid tumors, and to identify a recommended dose and schedule. **Secondary Objectives:** To assess the antitumor activity (e.g., Objective Response Rate, Duration of Response, Progression-Free Survival) and immunogenicity of SGN-PDL1V.

2.2 Background & Rationale To evaluate PF-08046054/SGN-PDL1V, a novel, investigational vedotin antibody-drug conjugate (ADC) that delivers the cytotoxic agent monomethyl auristatin E (MMAE) to cells expressing programmed cell death ligand 1 (PD-L1). This trial addresses the high unmet medical need for patients with metastatic, relapsed, or refractory solid tumors who have failed standard-of-care therapies. SGN-PDL1V exhibits multiple mechanisms of action, including MMAE-directed cytotoxicity, a bystander effect, and immunogenic cell death, establishing a strong rationale for its evaluation in PD-L1 expressing solid tumors.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm (Five study parts: A, B, C, D, and E) **Blinding:** Open-label **Randomization:** N/A **Study Parts:** * **Parts A & B:** PF-08046054/SGN-PDL1V monotherapy dose escalation and optimization. * **Part C:** PF-08046054/SGN-PDL1V monotherapy dose expansion. * **Parts D & E:** PF-08046054/SGN-PDL1V given in combination with pembrolizumab (dose escalation and expansion).

3.2 Planned Enrollment & Duration Target **NS:** 315 subjects **Number of Sites:** Multicenter **Trial Status:** Recruiting **Estimated Primary Completion:** Follow-up extending up to 3 years

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Adult patients (≥ 18 years) with histologically- or cytologically-confirmed metastatic or unresectable solid tumors. Eligible tumor types include: * Non-Small Cell Lung Cancer [NSCLC] (excluding those with a neuroendocrine component or histology) * Head and Neck Squamous Cell Carcinoma [HNSCC] (except nasopharyngeal cancer) * Esophageal Squamous Cell Carcinoma [SCC] * Triple Negative Breast Cancer [TNBC] * Melanoma * Ovarian Neoplasms/Cancer * Gastric Cancer 2. Participants must have disease that is relapsed or refractory, has progressed on approved therapies, or be intolerant to/refused such therapies. 3. Eastern Cooperative Oncology Group (ECOG) performance status of 0 or 1. 4. PD-L1 expression based on historical testing (e.g., $\geq 1\%$ by Combined Positive Score (CPS) or Tumor Proportion Score (TPS), depending on the study part and indication). 5. Measurable disease per RECIST v1.1 at baseline.

4.2 Exclusion Criteria 1. History of another malignancy within 3 years of first dose of study treatment or any evidence of residual disease from a previously diagnosed malignancy. 2. Known active central nervous system (CNS) metastases. Participants with previously-treated brain metastases may participate provided they are clinically stable for at least 4 weeks, have no new or enlarging metastases, and are off of corticosteroids prescribed for symptoms associated with brain metastases for at least 7 days prior to first dose. 3. Lepto-meningeal disease. 4. Prior treatment with an anti-PD-L1 agent within less than 5 half-lives. 5. Previous receipt of a monomethylauristatin E (MMAE)-containing agent. 6. Pre-existing neuropathy $\geq$ Grade 2 per National Cancer Institute Common Terminology Criteria for Adverse Events (NCI CTCAE) v5.0.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: PF-08046054/SGN-PDL1V given via intravenous infusion. Evaluated as monotherapy (Parts A, B, and C) and in combination with pembrolizumab (Parts D and E). **Route of Administration:** Intravenous (IV) **Duration of Treatment:** Until disease progression, unacceptable toxicity, or completion of up to 3 years of safety and efficacy evaluation.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care for oncology patients. **Prohibited:** Corticosteroids prescribed for symptoms associated with brain metastases within 7 days prior to the first dose; overlapping investigational or targeted anti-cancer agents.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, baseline scans, PD-L1 tumor status check, and eligibility assessments.
Baseline	Day 0	First Dose, safety monitoring, PK profiling.
Interim	Cycle 1 and beyond	IV infusions on designated cycle days; Safety labs, tumor imaging every 8-12 weeks.
End of Study	Up to 3 Years	Final safety evaluations, survival follow-up, and End of Treatment (EOT) review.

7. Outcome Measures (Endpoints)

7.1 Primary Endpoints 1. Number of participants with adverse events (AEs). 2. Number of participants with laboratory abnormalities. 3. Number of participants with dose-limiting toxicities (DLTs).

7.2 Secondary & Exploratory Endpoints 1. Confirmed objective response rate (ORR) per RECIST v1.1 by investigator assessment. 2. Duration of objective response (DOR) per RECIST v1.1 by investigator assessment. 3. Progression-free survival (PFS) per RECIST v1.1 by investigator assessment.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection evaluating safety profiles closely, with specific monitoring for anticipated ADC toxicities such as peripheral sensory neuropathy, diarrhea, fatigue, and neutropenia/anemia. **Serious Adverse Events (SAEs):** Must be reported to the sponsor within 24 hours of investigator awareness. **Data Monitoring Committee (DMC):** Evaluates dose-escalation to recommend dosing for optimization and expansion cohorts.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: * Safety Analysis Set (all patients receiving ≥ 1 dose of study drug). * Efficacy Evaluable Set (patients with measurable disease at baseline and at least one post-baseline scan). **Sample Size Justification:** 315 subjects targeted empirically across five study parts to detect safety signals, assess laboratory abnormalities, establish DLTs, and determine preliminary efficacy accurately. **Handling of Missing Data:** Efficacy missing data is censored at the last evaluable time point per RECIST v1.1. Safety data is summarized descriptively.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Standard clinical site monitoring and source data verification will be conducted periodically. **Audit Trail:** Secure database utilization for eCRF locking and standardized data clarification systems.

11. Study Identifiers & Governance

Primary ID: C5851001 (SGNPDL1V-001) **Registry Number:** NCT05208762

Sponsor/Company: Pfizer **Study Title:** A Study of PF-08046054/SGN-PDL1V in Advanced Solid Tumors **Official Regulatory Title:** A Phase 1 Study of PF-08046054/SGN-PDL1V in Advanced Solid Tumors **Trial Status:** Recruiting

12. Clinical Framework (Oncology Specific)

Disease Areas/Conditions: Carcinoma, Non-Small-Cell Lung Carcinoma, Squamous Cell Carcinoma of the Head and Neck, Esophageal Squamous Cell Carcinoma, Ovarian Neoplasms, Melanoma, Triple Negative Breast Neoplasms, Gastric Cancer. **Diagnosis Threshold:** Histologically or cytologically confirmed metastatic or unresectable disease. **Medical Methodology:** Targeted Antibody-Drug Conjugate (ADC) therapy (alone and with pembrolizumab). **Optimization Target:** MMAE-directed cytotoxicity in PD-L1 expressing tumors.

13. Study Procedures & Methodology

Management Pathway: Standard oncology treatment and clinical trials pathway. **Education Protocol (HCP):** Site Initiation Visits (SIV) and Investigator meetings focusing on ADC toxicities (e.g., peripheral neuropathy management). **Education Protocol (Patient):** Informed consent process detailing the phased trial approach and frequent PK/PD blood sampling. **Quality Audit Mechanism:** RECIST v1.1 central/investigator tracking and rigorous independent safety monitoring.

14. Observation & Follow-Up Schedule

Total Duration: Up to 3 years **Onsite Visit Cadence:** Per designated cycle schedule **Remote Monitoring Frequency:** Intermittent per site procedures for survival follow-up post-treatment **Checklist Review Interval:** Tumor imaging assessments every 8 to 12 weeks during treatment

15. Sign-off & Approval

Role	Name	Signature	Date		:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]					